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DEVELOPMENT OF A REGIONAL NUTRITION POLICY FOR ARKHANGELSK

Report on a WHO Workshop

Arkhangelsk, Russian Federation
22–24 November 1999

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EUROPEAN HEALTH21 TARGET 11

HEALTHIER LIVING

By the year 2015, people across society should have adopted healthier patterns of living

(Adopted by the WHO Regional Committee for Europe at its forty-eighth session, Copenhagen, September 1998)

ABSTRACT

The process of developing a healthy nutrition policy in the Russian Federation began in 1998 on the initiative of the WHO Regional Office for Europe and with the support of the Norwegian Government. It is being coordinated by the State Research Centre for Preventive Medicine of the Russian Ministry of Health and the Research Institute of Nutrition of the Russian Academy of Medical Sciences, with the aim of developing guidelines for a regional healthy nutrition policy specifically for the north-western part of the Russian Federation. In November 1999 a Workshop was held in Arkhangelsk to continue the drafting of these guidelines and to develop a policy of healthy nutrition for women and their families in Arkhangelsk. Participants made recommendations to authors regarding the chapters, timeframe for completion and revised contents list of the guidelines. The nutrition situation and opportunities for implementing a healthy nutrition programme for women and their families in Arkhangelsk were discussed at a meeting in the city mayor's office attended by relevant officials and representatives of WHO, the Research Centre for Preventive Medicine, the Institute of Nutrition and the regions. The Deputy Mayor of Arkhangelsk expressed his appreciation of the work being carried out and said that the city administration would support it.

Keywords

NUTRITION POLICY
MATERNAL WELFARE
CHILD WELFARE
FAMILY HEALTH
REGIONAL HEALTH PLANNING
RUSSIAN FEDERATION

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Foreword

Healthy nutrition for women and their families

It is widely recognized that interventions directed at women's health are highly cost-effective. This report describes an event concerning women which is part of an ongoing process in the Russian Federation, notably in the north-west, thanks to support from the Norwegian Government. The process of developing healthy nutrition policy for regions in the Russian Federation is particularly timely because of the economic problems suffered by the population. Calculations by the World Bank show that improving the health of women offers the greatest return on investment and that there are multiple payoffs in terms of wellbeing and significant benefits for families, communities and national and local economies and thus the population as a whole (see the World Bank's web-site www.worldbank.org/html/extdr/hnp/health/newagenda/women.htm, – describing its agenda for women's health and nutrition).

The World Bank recommends the promotion of positive health practices, including access to information about nutrition and public health. The focus on women is especially relevant in the Russian Federation because it is generally the women who take care of families and the household. The major decisions made concerning the purchase and preparation of food is controlled by women, so that women are the key to improving the nutritional status of the population. Women are also the key to implementing the concept of food-based dietary guidelines to prevent diet-related chronic noncommunicable diseases (NCD) such as cardiovascular disease, the main cause of premature mortality in the Russian Federation today.

Life-cycle approach to public health policy

Life contains a series of critical periods which are marked by particular events. Some circumstances are common to everyone: birth; growth and development; acquiring basic social and health skills; entering school; transition from primary to secondary education; puberty and adolescence; school examinations; responsibility in society; entering the labour market; establishing a home; job security; parenting; caring for a family; caring for parents; children leaving home; onset of chronic disease in family and/or friends; leaving employment and retirement; aging; loss of spouse, family and/or close friends; death.

Some life events are specific to women, notably childbirth; multiple jobs and roles (paid and unpaid); gynaecological- and obstetric-related health problems and menopause. Women may suffer from more human rights abuse and from inequalities in society and so may be in a relatively vulnerable position. On the other hand, as mentioned above, if women are encouraged and supported through specific programmes, the investment is likely to reap dividends.

Sound investment for the whole population

Healthy nutrition policy is a good example of a strategy that will yield a benefit from investing in women. In addition to deciding what food is bought, prepared and served to the family, the woman may also play a key role in growing food if the family have access to even a small area of land. Moreover, it will be her choice whether her children are breastfed exclusively for about the first six months and she will decide what they eat and how they are fed after six months.

Recent scientific evidence has proved that nutrition during the early years is a key determinant of health and the development of NCD during adult life. Therefore, there are many convincing health, social and economic reasons why women should be targeted. They can help to change the eating habits, not only of their families, but also of society and the Russian population.

Prevention of disease

Building the self-esteem of women, promoting breastfeeding and implementing correct feeding practices for infants from six months to three years of age are especially important. Building self-esteem is crucial so that young girls and adolescents develop a “healthy” body image and stay away from distorted dieting practices (leading to disorders such as anorexia), and perhaps more important delay teenage pregnancy, which is associated with increased health risks for both mother and child. It has been clearly shown that diet during intra-uterine growth of the foetus during pregnancy and feeding practices during the first years of life have a dramatic impact on the development of diet-related chronic NCD later in life. This is a major reason why it is important to focus on women and children as a way of reducing the prevalence of NCD in adults in the Russian Federation.

Childhood malnutrition flourishes during periods of vulnerability and newborns of low birth-weight are at especially high risk of morbidity and mortality because of their rapid growth. As adults, they tend to be smaller by 5 cm in height and 5 kg in weight, with a reduced work capacity, IQ and strength. A period of relative wellbeing can follow after birth, even in settings of marked poverty – but only if infants are exclusively breastfed for about six months. In infancy growth rates begin to falter dramatically if optimum nutrition is not provided, and particularly before one year of age. By the time poorly-fed children are two or three years many will be irreversibly stunted.

From the age of three years until starting school, most children, even if poor, will grow generally as well as those that are better-off, remaining small but neither falling further behind or catching up appreciably. Some catch-up occurs in some settings during adolescence but at present there is no evidence that stunting can be reversed. Thus to prevent poor nutritional status, especially stunting and its long-term consequences (see next paragraph), efforts must be made to promote good growth and development in the first two to three years of life.

Children under three years are most vulnerable to poor nutrition because the growth rate in the first years is higher than at any other time. Thus adverse factors such as poor nutrition have a greater potential for causing stunting. The immunological system only matures with time and so the young are also more susceptible to frequent and severe infections. Cognitive and emotional abilities develop in the early years, thus the foundation of intellectual, social and emotional skills are established during this period. Poor nutrition both during pregnancy and the early years leads to stunting (growth retardation) which is associated with delayed motor and cognitive development (resulting in low IQ), a greater degree of behaviour problems, deficient social skills at school as well as decreased attention span, deficient learning and lower educational achievement, and an increased risk of NCD in adult life.

Obesity is another major risk factor for NCDs. Its prevalence is extremely high in the Russian Federation, especially among women: approximately 22% of women are obese, compared with far fewer in the Nordic countries, for example 10% in Finland. It has been shown that the promotion of exclusive breastfeeding helps to reduce the prevalence of obesity in women and in their offspring who are lucky enough to be breastfed. Later on obesity can be prevented by

increased physical activity. The skills needed to establish healthy nutrition habits and increased physical activity should be taught during childhood to boys and girls in school, although girls are most likely to implement these skills afterwards when they have families.

Health promotion

Having agreed the need to target nutrition policy through women, there is a need to prepare women to be able to promote both their own and their families' health. Appropriate interventions must be developed to demonstrate how this can be achieved. For example, adopting a life-course approach to nutrition policy acknowledges the complex interactions between life events, biological roles and associated risks, and their health consequences. At each critical point in the life cycle, supportive actions are needed at both macro- and micro-level. Macro-interventions include integrated social, economic and other public policies to create supportive environments to promote the full health potential of society. Micro-actions are closer to the family or community, school or workplace. This concept is the same as that on which the WHO countrywide integrated noncommunicable diseases intervention (CINDI) programme network is built, which promotes integrated interventions to prevent disease and promote health. It is therefore excellent and a great achievement that Arkhangelsk, following this meeting and thanks to the investment of Norway, has agreed to join the CINDI network in the Russian Federation.

A health promotion and disease prevention approach will encourage the integration of many determinants of health and aims to empower people, especially, in this case, women. Building public health policy (Towards a Healthy Russia, Moscow 1997) includes creating supportive environments, strengthening community action, developing personal skills for healthy lifestyles and reorienting health services to be less therapeutic and more prevention-minded.

Applying this to nutrition policy, it can be seen that addressing women helps to build public health policy. An adequate food supply and access to it (both logistically and financially) is critical for a supportive environment which provides good nutrition. Community action can be strengthened through antenatal health programmes targeted at pregnant women and mother support groups. Other community-based initiatives can be designed both to promote health and to address the needs of women who are going through difficult times during the economic transition in the Russian Federation. Developing their skills includes strengthening their ability to make life choices and build a civil society in a fledgling Russian democracy. This is especially relevant as regards which foods are produced and sold in local retail outlets in Arkhangelsk in the new market economy system.

Concepts such as advertising and marketing were unknown in the former Soviet Union, so people need new skills to ensure that they are not exploited by profiteers. Information campaigns, based on healthy eating principles, should be directed through the media (newspapers, women's magazines, television) and schools. All local authorities can play a role in buying healthy food and providing information about healthy nutrition for citizens.

More emphasis should be placed on the need for health services to develop good counselling services through the existing primary, secondary and tertiary health care system. Health professionals (especially those dealing with women and children) should be able to give nutritional recommendations and dietary guidance based on modern scientific principles. This information should be readily available free from all health care departments. In addition, perhaps through the sanitary-epidemiology system, nutrition and health surveillance should be developed in order to facilitate a health information system. This health information system can

help to inform policy and policy-makers about the appropriate strategies and identify those that are most effective in promoting health.

Local mechanisms and infrastructures

In order to develop and monitor the implementation of policy it is vital that a mechanism and implementing infrastructure is established. For example, a local food and nutrition council should be formed in Arkhangelsk with the capacity to advise policy-makers such as mayors and local politicians. The information collected via the health information system can be fed into this council and strategic decisions on how best to develop policy and implement an action plan to achieve the desired results can be taken.

Enhanced human capital through improved nutrition and health is thought to explain half of the economic growth of France, the United Kingdom and other European nations during the last two centuries. The rise in the prevalence of diet-related diseases is likely to be “programmed” during development of the foetus and early childhood (1–3 years) and these problems have functional consequences and limit the potential of a population. As Russian regions face difficult choices during times of economic crisis, it becomes imperative to advocate strongly for investment in the social sector, including in healthy nutrition policy for women and their families.

In conclusion, there is a long way to go but the experts in Arkhangelsk have made excellent progress. The process described in this report has got off to an excellent start but of course resources and human capacity are needed to maintain the progress to date. It is therefore particularly impressive that the Arkhangelsk local authorities agreed in December to give political support to developing healthy nutrition policy.

The blessing of the local authorities means that any changes made from now on are likely to be sustainable in the long term. A sound investment indeed!

And congratulations to all those who made it happen.

Dr Aileen Robertson
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Executive summary

The process of developing a healthy nutrition policy in the Russian Federation began in 1998 on the initiative of the WHO Regional Office for Europe and with the support of the Norwegian Government. It has been jointly coordinated by representatives of the State Research Centre for Preventive Medicine of the Ministry of Health of the Russian Federation and the Research Institute of Nutrition of the Russian Academy of Medical Sciences. The purpose of this cooperation is to develop guidelines for a regional healthy nutrition policy specifically for the north-western part of the Russian Federation. Several regions in this area have formed a coalition for this purpose, including Arkhangelsk, Murmansk, Moscow, Krasnodar, Perm, Chelyabinsk and Electrostal (the Moscow Region).

Several meetings and workshops have been conducted during this time to develop a healthy nutrition policy. Needs assessments have been conducted among the general public and among representatives of various institutions dealing with nutrition issues, and official statistics and available data of the studies of the population dietary patterns have been collected and analysed. Regional representatives were asked to draft their proposals and write their respective chapters of the guidelines in time for this meeting in Arkhangelsk.

In parallel with the drafting of the guidelines, local coalitions have been formed to develop regional programmes for the north-west. Thus, such a coalition has already been established in Chelyabinsk, while in Arkhangelsk the Medical Academy, in collaboration with the Centre of the State Sanitary and Epidemiological Surveillance, has initiated the development of a healthy nutrition programme for the region.

In September 1999, a workshop on healthy nutrition and a health policy for women and their families was conducted in Murmansk. It was at that workshop that initiatives were taken to draft a plan of action for nutrition for the Murmansk Region, and a working group was set up to develop it.

The aim of the Workshop in Arkhangelsk in November 1999 was to develop a healthy nutrition policy for women and their families in Arkhangelsk and to continue drafting the guidelines for a regional healthy nutrition policy in the Russian Federation.

The first two days of the Workshop were devoted to progress on development of the guidelines for a regional healthy nutrition policy in the Russian Federation. Working materials or draft chapters of the document had been prepared for the meeting. In some instances, there were several versions of chapters. The Workshop resulted in recommendations to authors regarding chapters, a timeframe for completion and a revised contents list.

The last day of the Workshop was devoted to a meeting to discuss the nutrition situation and opportunities for implementing a healthy nutrition programme for women and their families in Arkhangelsk. This meeting was attended by officials from the city mayor's office, heads of the city's various departments and services, and representatives of WHO, the Research Centre for Preventive Medicine, the Institute of Nutrition and the regions. The Deputy Mayor of Arkhangelsk expressed his appreciation of the work being carried out and assured the meeting that the city administration will support it.

Background to the development of a regional healthy nutrition policy in the Russian Federation

The process of developing a regional healthy nutrition policy in the Russian Federation began in 1998 on the initiative of WHO. It has been jointly coordinated by representatives of the State Research Centre for Preventive Medicine (RCPM) of the Ministry of Health of the Russian Federation and the Research Institute of Nutrition of the Russian Academy of Medical Sciences, with the aim of developing guidelines for a regional healthy nutrition policy in the Russian Federation. Several regions in this area have formed a coalition for this purpose, including Arkhangelsk, Murmansk, Moscow, Krasnodar, Perm, Chelyabinsk and Electrostal (the Moscow Region). Several meetings and workshops have also been conducted with the aim of developing a healthy nutrition policy.

Needs assessment surveys have been carried out among the general public and among representatives of various institutions dealing with nutrition issues, and official statistics and available data of the studies of the population dietary patterns have been collected and analysed. The possible contents of guidelines for a regional healthy nutrition policy were discussed at a meeting in Chelyabinsk in May 1999 (see the report of the meeting). Members of a working group, which includes representatives of the regions, were asked to draft their proposals and write their respective chapters of the guidelines in time for the present meeting.

In parallel with the drafting of the guidelines, local coalitions have been formed to develop regional programmes for the north-west. One such, in Chelyabinsk, includes representatives of various institutions – the administration, health and education services and the mass media. In Arkhangelsk, the Medical Academy, in collaboration with the Centre of the State Sanitary and Epidemiological Surveillance, has initiated the development of the regional healthy nutrition programme, and a document on healthy nutrition for women and their families in Arkhangelsk for 2000–2002 has been drafted (Annex 1).

In September 1999, a workshop on healthy food and nutrition and health policy for women and their families was conducted in Murmansk, with the aim of providing health professionals with practical and evidence-based information on maternal nutrition and breastfeeding support. It was at that workshop that activities were started to draft a plan of action for nutrition in the Murmansk Region, and a working group was set up to develop it (see the report on the workshop).

Aim and objectives of the Workshop

The aim and objectives of the Workshop were:

1. to continue the drafting of the guidelines for a regional healthy nutrition policy in the Russian Federation;
2. to develop a policy of healthy nutrition for women and their families in Arkhangelsk.

Discussion of the draft guidelines for a regional healthy nutrition policy in the Russian Federation

Working materials and draft chapters of the guidelines for a regional healthy nutrition policy in the Russian Federation had been prepared for the meeting (Table 1). In some instances, there were several versions of chapters.

Table 1. Materials submitted

Author	Materials submitted	No. of pages
Baturin A.K.	Nutrition and health of the population of the Russian Federation. Nutrition of the population of the Russian Federation.	14
Vartapetova N.V.	Infant feeding during the first year: experience and prospects for the development of breastfeeding promotion programmes (summary of the chapter). Experience of healthy nutrition programmes in the Russian Federation and abroad (summary of the chapter).	4
Volkova E.G., Levashov S.Yu.	Experience of developing a regional healthy nutrition policy in the Chelyabinsk Region. Sections: introduction, problem identification, assessment of the nutrition situation, improving food security of the population, protection of the population through ensuring food safety and quality, promotion and support of breastfeeding, caring for the deprived and vulnerable, micronutrient deficiencies, appropriate diets and lifestyles, education and nutrition, research on nutrition, international cooperation, coalition building.	33
Glazunov I.S.	Introduction. Why a healthy nutrition policy is needed.	2
Danchin N.M., Karabak T.I.	Problems of healthy nutrition of the population in the Krasnodar Territory and ways of resolving them. Sections: assessment of the quality of the diets of the population and the operation of the regional food industries; prospects for developing and implementing a healthy nutrition policy; the soya processors' association as a partner in implementing the healthy nutrition policy; environmental monitoring; conclusions.	8
Kalev O.F.	Training health professionals in healthy nutrition. Regional opportunities for teaching the principles of healthy nutrition to the public. Special features of developing a healthy nutrition policy for various populations. Annexes: a table of serving sizes, glossary of terms; optimal nutrient intake in a population, WHO objectives in the field of nutrition; references.	19
Kamardina T.V.	Healthy nutrition marketing at the regional level. Nutrition monitoring and assessment.	3
Konstantinova S.V.	Special features of developing a healthy nutrition policy for various populations. Public catering.	13
Koudrya L.I., Sokolova L.A., Kondakova N.I.	The Programme "Healthy nutrition of the population of Arkhangelsk for 2000-2002."	14
Lebedeva T.M., Brazhkin A.V., Khoroshavin V.A., Akimova L.N.	Sections suggested for inclusion in the draft programme "Regional policy of healthy nutrition of the population of the Russian Federation": introduction; Perm's experience and programme of improvements: food production and processing, quality and safety of food raw materials and food, trade and labelling, public catering, conclusions and recommendations; experience of surveying schoolchildren's diets and the programme of improving students' diets: conclusions and recommendations; references.	23
Pestoun L.M., Zubareva N.L., Davdenko N.T., Romanchouk L.I., Shmarova L.M.	Healthy diet in the school of diabetes mellitus (the experience of the school of diabetes in Electrostal, Moscow Region). Experience of surveying schoolchildren's diets and the programme of improving students' diets. Experience of developing and implementing a policy of breastfeeding promotion in Electrostal, Moscow Region.	29
Popovich M.V.	Assessing the needs for the development of a healthy nutrition policy for the population. Principles of healthy nutrition.	5

The work of the meeting was conducted in plenary sessions, where general issues regarding the writing of the guidelines were discussed, and in working groups. Participants considered the possibility of the guidelines being given official status so that they would be binding on the Russian Ministry of Health. However, they agreed that although this would raise the status of the guidelines, it would also significantly increase the time needed to develop them. It is now more important to build coalitions at the local level and to secure the interest and involvement of institutions dealing with nutrition policy. Participants decided that the guidelines should be of an advisory nature and include recommendations for the regions. At the same time, they decided that they should contain letters from the top officials of the Ministries of Health, Education and Agriculture and the State Duma, and a recommendation from WHO to implement the guidelines.

Participants agreed that the guidelines should not be a strictly health care programme. Other sectors, such as education, trade and agriculture, should also be involved in the drafting and implementation of the guidelines.

The regional character of the guidelines was emphasized: they will be written first and foremost for the regions. Many issues can and should be resolved at local level – a number of issues cannot be resolved or even discussed at the federal level. This is also important from the point of view of funding. It is easier to find resources at the local level.

Participants discussed, and agreed, the concept of healthy nutrition as the only term in the document characterizing appropriate dietary patterns. Such terms as balanced nutrition and rational diet should be viewed as part of the concept of “healthy nutrition”. This definition should be included in the guidelines.

Participants suggested that annexes should be included in the guidelines, although they decided not to discuss the materials to be included in the annexes at this stage. These should be considered after the completion of the guidelines.

The authors of chapters were asked to submit their materials by 10 December 1999. The RCPM would process the material within one week and prepare the first draft of the document to be circulated among the regions for comment. By the end of December, the draft document should be ready to be sent to WHO.

Participants discussed the contents of the document and agreed that materials prepared for some of the chapters should be moved or altered, for example:

- the chapter on the features of the policy for different populations will be merged with another chapter;
- the chapter on marketing should be merged with the chapter on public awareness;
- a section should be added on the role of various health services and on breastfeeding;
- complementary feeding as one of the strategies will be added;
- a few subsections on the situation analysis should be added to section 4.

The revised and agreed table of contents of the document is given in Annex 4.

The materials listed below were discussed in working groups and participants agreed on further work and on a time-schedule (Table 2).

Table 2. Schedule of work on the guidelines in December 1999

Stages of work	1 st week	2 nd week	3 rd week	4 th week
1. Writing chapters along the lines recommended by this meeting				
2. Chapters to be received by the Research Centre for Preventive Medicine (RCPM)	10 December, 1999			
3. Discussion of the chapters and editing by the editorial group at the RCPM and the Institute of Nutrition				
4. Mailing of the first version of the guidelines to WHO	20 December, 1999			

Recommendations for authors of chapters of the guidelines

The working group for the drafting of the guidelines made the following recommendations concerning the chapters.

Chapter 1. Introduction (Mr. I. Glasunov's version)

The author should include references to the CINDI programme documents, such as the protocol, reports on discussions of a regional healthy nutrition policy in Chelyabinsk, Murmansk and Arkhangelsk, as well as the document "Towards a healthy Russia", the policy and strategy of preventing CVD and other noncommunicable diseases in the context of the health care reforms." The title "Regional policy" should have the subtitle "Guidelines" to be used throughout the text of the introduction. Reference to women and the lifecycle approach should be included.

Chapter 2. Why a policy of healthy nutrition is needed (Mr. I. Glasunov's version)

The author should add paragraphs on the importance of nutrition for health and on today's problems in the field of nutrition, and then briefly outline actions to be taken to resolve the problems so that preventive measures could be planned on the basis of these data. A diagram on the lifecycle approach should be included.

This section should contain an explanation that the working group has defined the notion of "healthy" nutrition as rational and balanced nutrition. These words are synonymous. Since these synonyms are used in a number of documents on nutrition in different senses, this chapter should provide a definition of "healthy nutrition". Healthy nutrition is nutrition that provides for the child's growth and normal development, strengthens health, prevents diseases and ensures a long life.

Chapter 3. International experience of healthy nutrition programmes (N. Vartapetova)

The author should add the following:

- the European Plan of Action, its development and the parallel process of developing the regional healthy nutrition policy in the Russian Federation that takes into account the provisions of the European Plan;
- current Russian experience with elements of a healthy nutrition policy – salt iodization, healthy nutrition of young children, nutrition in institutional populations; breastfeeding, food safety – presented either at federal or regional levels;

- an emphasis that professionals are still being trained as therapeutic nutrition specialists, not as specialists in healthy nutrition;
- international experience, including the experience of Finland and North Karelia, with a description of each of the programmes and a summary of the main results;
- a brief subsection with conclusions.

Items 1 and 2 of the short table of contents of the chapter should be omitted.

Chapter 4. Situation analysis and needs assessment for the development of a policy of healthy nutrition of the population (A. Baturin, M. Popovich, S. Levashov)

An introduction should be provided and a description given of the problem analysis in the field of nutrition, indicating what data and studies it includes and why it is necessary.

Chapter 4.1. Trends in morbidity and mortality. Their relation to nutrition and women and children (A. Baturin)

The author should:

- show the connection between nutrition and various diseases, indicating the contribution of nutrition to morbidity, and micronutrients and their relation to diseases;
- include the diagram from the European Plan of Action showing the role of nutrition for humans;
- provide data on cholesterol and excessive body mass as risk factors for various diseases and their relation to mortality;
- standardize data provided on mortality due to the major causes by age to ensure comparability with other regions of the Russian Federation and with data outside the Russian Federation; provide regional mortality data;
- replace all tables with charts.

Chapter 4.2. Nutrition of women and children in the Russian Federation (A. Baturin)

The author should include material from Chelyabinsk, Krasnodar, Arkhangelsk, Elektrostal and Perm in this section.

The author should also:

- provide data on iodine and fluorine intake based on various studies;
- show the prevalence of cholesterol and excessive body mass; present the conclusions of the chapter more clearly;
- provide, as a separate subsection in this section, data on food quality and safety but reduce the number of tables and give them as short examples;
- include data on trade in the streets and market-places, to demonstrate violations of food safety requirements;
- include a separate subsection on public catering which should contain data on the studies of public catering;

- provide data on breastfeeding available in the Russian Federation, including experience in Electrostal, and the economic benefits of the programme;
- include complementary feeding and previous Soviet recommendations;
- emphasize that the main direction in healthy nutrition should be the need to resolve problems with nutrition by changing diets and through healthy nutrition, and that a recommendation to use biologically active dietary supplements would be dangerous since people often prefer to resolve problems of nutrition through the use of tablets.

Chapter 4.3. Assessment of the needs for the development of a policy of healthy nutrition of the population (M. Popovich and S. Levashev)

This chapter should be revised by Dr Levashov so that:

- it is more structured, with subsections, and
- it includes an explanation as to why the needs are to be studied.

Chapter 5. Recommendations for the development of a regional policy of healthy nutrition of the population

Chapter 5.1. Principles of healthy nutrition (M. Popovich)

The author should:

- include the recommendations for healthy nutrition used in the CINDI healthy nutrition guidelines;
- present the WHO healthy nutrition pyramid, without describing the quantities of food;
- give recommendations of the 12 steps with short commentaries;
- include the hip–waist ratio;
- show how failure to observe the principles of healthy nutrition leads to obesity and other problems.

Chapter 5.2. Coalition. Reaching consensus. Building partnership (I. Glasunov)

The author should:

- include in this section materials on partnership from the section on marketing and from the section on the special features of various populations dealing with partnership;
- provide the patterns of relationship with the stakeholders involved in the nutrition policy, and describe the role of each stakeholder.

Chapter 5.3. Laws and regulations, taking into account regional and ethnic features (T. Kamardina, L. Sokolova, A. Brazhkin)

The authors should:

- describe the current status of the legal framework (some material has been provided by A. Baturin);

- articulate the provisions that are needed to develop and implement a healthy nutrition policy;
- give examples of regional regulations in Krasnodar and Arkhangelsk;
- emphasize that not all can be done at the national level – often, more effective results can be achieved at the regional level;
- mention the law on the vitamin fortification of bread as an example.

Chapter 5.4. The role of health services (N. Danchin, L. Pestoun, A. Brazhkin, L. Sokolova)

The authors should mention the role of health services in initiating a healthy nutrition policy. This section should describe the roles of and provide recommendations for:

- the primary health care service (L. Pestoun, N. Danchin);
- medical prevention centres (N. Danchin);
- the state sanitary and epidemiological surveillance service (A. Brazhkin, L. Sokolova).

Chapter 5.5. Training professionals in healthy nutrition (O. Kalev, E. Volkova)

The authors should:

- include issues relating to the training of other professionals, such as educationalists and people working in public catering, administration and trade;
- discuss the possibility of creating a new type of professional for public catering;
- put forward a recommendation to stop using the notion of therapeutics and dietetics alone and in addition to promote public health nutrition;
- reduce the size of the section.

Chapter 5.6. Promotion of healthy nutrition principles among the general public. Public awareness (O. Kalev, T. Kamardina)

The authors should:

- combine the section on marketing with this section, and
- include recommendations regarding channels of dissemination of public information.

Chapter 5.7. Nongovernmental consumers' organizations (L. Sokolova, M. Popovich)

This chapter should deal with the protection of consumers' rights in the production, storage, transportation and marketing of food and in public catering, and the opportunities for involving nongovernmental organizations in raising public awareness.

Chapter 5.8. Breastfeeding and complementary feeding (N. Vartapetova, L. Pestoun)

The authors should present this issue as one of the strategies of healthy nutrition, and formulate recommendations for implementing this strategy in the Russian Federation.

Chapter 5.9. Agriculture and the healthy nutrition policy (I. Glasunov, A. Baturin)

Recommendations for the agro-industrial complex will be formulated in consultation with representatives of this sector after the opportunities for implementing them have been discussed. The Netherlands is a good example.

Chapter 5.10. Food production and processing. Storage. Quality and safety of the raw materials for food and final food products (L. Sokolova, A. Brazhkin)

Recommendations regarding these issues will be formulated by officials of the State Sanitary and Epidemiological Surveillance and discussed with representatives of the institutions concerned, focusing on the opportunities for implementation.

Chapter 5.11. Trade. Labelling (L. Sokolova, L. Koudrya)

Recommendations for the trade sector will be formulated in consultation with representatives of this sector, and opportunities for their implementation will be discussed.

Chapter 5.12. Public catering (M. Shraga, L. Koudrya, S. Konstantinova)

This chapter should be rewritten. Data on studies characterizing public catering should be presented in Chapter 4 on the situation analysis. This section should provide recommendations for institutions with public catering facilities (children's pre-school institutions, schools, cafés, canteens, restaurants, hospitals, prisons).

Chapter 6. Monitoring and evaluation (T. Kamardina)

The author of this chapter should:

- define more precisely the ultimate objectives of the programme, present the diseases caused by inappropriate nutrition, and show the possibility of short-term evaluation, and
- present the evaluation procedure.

Discussion of healthy nutrition for women and their families – a regional policy approach for Arkhangelsk

On the last day of the workshop a meeting was held to discuss the nutrition situation and the opportunities for implementing a healthy nutrition policy in the city of Arkhangelsk, attended by officials of the city mayor's office and heads of the various departments and services in the city. The programme and the list of participants of the meeting are given in Annexes 2 and 3, respectively.

Dr Aileen Robertson presented the first Food and Nutrition Action Plan for the WHO European Region for 2000–2005, which had been developed by the WHO Regional Office for Europe. It is important that the regional healthy nutrition policy in the Russian Federation is formulated in parallel with this Action Plan, which largely facilitates its development.

WHO points out that inequity in food consumption and the lack of safe healthy food is responsible for over 1 million deaths (14% of all deaths) in the WHO European Region among adults and children. Cardiovascular disease causes the death of over 1.4 million people every year. More than 30 million people in Europe are afflicted with heart diseases, resulting in the loss of millions of working days and in enormous costs incurred by the health services.

As with the Plan of Action against Smoking, ensuring access to healthy food is a major activity in the health sector which countries should undertake to improve public health and promote

economic growth. This activity will be part of the WHO HEALTH21 health for all policy framework for the WHO European Region.

It is extremely difficult to change the current nutrition stereotypes, characterized by a high intake of fats and high-energy foods, and to introduce new approaches. Nevertheless, through women and their motivation towards their families it is perfectly realistic to strengthen the strategies aimed at supporting breastfeeding of infants and increasing consumption of safe healthy food by women and children. Strategies include increased production and consumption of vegetables and fruit to reduce the risk of cardiovascular disease, prevent micronutrient deficiencies, promote growth and development and ensure safety of food.

Dr A. Baturin presented data on food consumption in the Russian Federation and the national concept of the policy of healthy nutrition of the population until 2005.

Dr M. Shraga presented nutrition-related morbidity rates in the population of the Arkhangelsk Region, and showed data on food consumption by adults and children. He also presented the Programme for a Healthy Nutrition Policy for Women and their Families in Arkhangelsk – a Regional Approach (Annex 1).

The Deputy Mayor of the city of Arkhangelsk expressed his appreciation of the work being carried out and assured the meeting that the city administration would support this work.

The Administration of the Arkhangelsk State Medical Academy called on the National Director of the National CINDI-Russia Programme to consider including the Arkhangelsk region in the CINDI programme, so as to facilitate the process of building a coalition in the city and promote exchange of experience between regional programmes in the Russian Federation.

Conclusions and recommendations

Participants in the Workshop discussed the materials that had been prepared and made the following recommendations:

- the guidelines for a regional healthy nutrition policy in the Russian Federation currently being drafted should be defined and referred to as the guidelines;
- the guidelines are recommendations, intended predominantly for the regions;
- a separate chapter on breastfeeding should be added to section 5;
- a chapter on the role of health services should be added;
- the chapter on teaching the principles of healthy nutrition to the public and the chapter on the healthy nutrition marketing should be combined into a new chapter entitled “Promotion of healthy nutrition principles among the general public. Public awareness”;
- the principles of healthy nutrition should be understood to embrace the WHO and CINDI recommendations on healthy nutrition and the 12 steps to healthy eating, as well as the healthy food pyramid.

Annex 1

Approved by Resolution of the Arkhangelsk
City Soviet of Deputies
No. _____ of _____ 1999

Programme for Healthy Nutrition Policy for Women and their Families – A Regional Approach for Arkhangelsk for 2000-2002

Background

Health indicators have deteriorated significantly in the city of Arkhangelsk in recent years.

Primary morbidity from diet-related diseases has increased considerably. In adults, there has been a 5 to 6-fold increase in the prevalence of blood and hemopoietic organ diseases: from 0.6 in 1991 to 3.6 cases per 1000 population in 1998. In this category, morbidity from anaemia has increased from 0.5 to 3.2 per thousand of population, respectively. During the same period, there was a 4-fold increase in the prevalence of anaemia in children (from 2.5 to 9.3 per 1000). In children under 1 year of age, the increase was from 151.7 to 253.4 cases per 1000 children during 1995-1998.

The prevalence of gastrointestinal disease increased three-fold in adults during the same period (from 13.4 to 36.2 per 1000, with the incidence of gastritis and duodenitis going up from 1.8 to 5.3 cases per 1000 adult population). In children the incidence increased from 68.8 to 91 cases per 1000. Primary morbidity from gastrointestinal disease among children rose by 33% compared to the year 1991. There has been a particularly dramatic increase in such nosological forms as peptic ulcer of the stomach and duodenal ulcer (6 times), gastritis and duodenitis (3 times).

The prevalence of rickets increased from 39.4 in 1995 to 78.2 in 1998.

The average annual rate of primary morbidity from neoplastic diseases in 1991-1998 in Arkhangelsk was 9.09%. The rates of incidence of cancer have been growing steadily over the past 15 years, from 225.1 to 245.6 per 100 000. The prevalence of breast cancer has increased from 26 to 37 per 100 000, that of colon cancer is now 9.1 per 100 000 in men and 14.7 in women. Colon cancer mostly afflicts women after 50.

Deterioration in the quality of nutrition is one the reasons for increasing morbidity of the public.

In Arkhangelsk only 28% of infants were breastfed for 6 months and longer; 38% of babies were only breastfed for 3, 4 or 5 months. The rate of unjustified introduction of artificial feeding or early introduction of mixed feeding (before 3 months) is extremely high – 34%.

Analysis of the diets of some population groups shows that their nutrition is not rational. The average daily food intake of children between 3 and 7 years of age in pre-school institutions in the city of Arkhangelsk continues to decrease, both in terms of the quantity and in terms of the variety of animal and vegetable products. The assortment of cereals, meat and fish products has reduced; instead, more canned meats, ribs, bones and other products rich in fat and poor in protein are consumed. The diet has virtually no fish (on the average, below 5 g per day) or dairy products; the amount of milk consumed per day has dropped to 180 g. There are practically no curds or cheese in children's diet. Vegetable intake has decreased dramatically, and fruit has practically disappeared from the diet in preschool institutions (on the average, not more than 5 g per day). Berries growing in the North are only consumed in small quantities because prices are high and supply through consumers' cooperatives is not possible.

A sharp imbalance of the major macronutrients is characteristic of the situation. Deficiencies of calcium, phosphorus and iron have been growing steadily. An increase in the prevalence of thyroid disorders called for supplying children's pre-school institutions with iodised salt for cooking using regional resources.

Children are largely deficient in vitamin C, the intake of which is 2-3 times below the reference value. Vitamin fortification of food in schools is not undertaken. Analysis of children's diet in schools shows that the organization and quality of nutrition in school canteens have deteriorated considerably. Although the canteens are adequately furnished with cooking and refrigeration electrical equipment enabling them to cook a variety of dishes in large quantities, they only cook food for some 20–30% of schoolchildren. According to a recent survey, only 17.5% of girls and 27.5% of boys in senior grades have breakfast at school every day. Between 20% and 40% of students never use the services of the canteen, while the rest have meals there occasionally. More than 15% of students do not like the food usually offered at school because of its monotony (40%) and poor taste (25%), which testifies to inadequate skills of the school canteen personnel and lack of proper control of the food quality on the part of the school administration. Analysis of the quality of school diets shows that 48% of children have tea with a bun for their school breakfast. The energy value of such a diet is 132-190 calories, which accounts for 5-7% of the daily energy requirement of children. 62% of children have breakfast containing not more than 6 g of protein. The school diet of 73% of children contains 3-4 g of fat, which is twice as low as the reference value. The average amount of carbohydrates is 27.3 g, or 8% of the daily requirement, while in 70% of students this amount is not more than 20 g. Such a diet is extremely poor in vitamins, minerals and trace elements. The content of vitamin C is 0.05 mg, while the daily requirement is 50 mg; that of calcium is 71 mg, or one-tenth of the requirement.

This programme has been developed and coordinated by the Arkhangelsk City Soviet of Deputies, the Department of Health and Social Protection of the population at the Mayor's office, the Centre of State Sanitary and Epidemiological Surveillance, Arkhangelsk State Medical Academy.

Those responsible for drafting the programme are: L. Koudrya, N. Kondakova at the Arkhangelsk State Medical Academy, and L. Sokolova at the Centre of State Sanitary and epidemiological Surveillance in Arkhangelsk.

The aim of the programme:

To reduce morbidity in the population from communicable and noncommunicable diseases, and mortality in children and adults.

The objectives of the programme:

- To provide rational and safe food.
- To promote healthy lifestyles of women and their families.
- To provide social protection for the low-income groups of the population and groups at risk (pregnant women, lactating mothers, children).

The time-frame for the implementation of the programme: 2000 – 2002.

Implementing agencies of the programme:

- The departments of the Mayor's Office: economics, industrial policy and investments; health and social protection of the population; education, culture and sports.
- The Committee for Trade and Public Services at the Mayor's Office of Arkhangelsk.
- The Financial Department at the mayor's office of Arkhangelsk.
- Arkhangelsk State Medical Academy.
- The Centre of State Sanitary and Epidemiological surveillance.
- Enterprises of all forms of ownership and legal organization, producing and marketing food.

Expected outcome:

Reduction in the prevalence of communicable diseases, and stabilisation of morbidity from noncommunicable diseases in children and adults.

The main criteria of evaluating healthy nutrition of the public:

- Increased consumption by the population of the basic food groups (cereals, vegetables, fruit, berries, dairy and fish products) recommended by the WHO.
- Reduced consumption by the population of animal fats and high-fat foods.
- Increase in the manufacturing and marketing of local foods.
- Supply of products correcting the deficiencies of trace elements (iodine, selenium, etc.) for the population of the city of Arkhangelsk.
- Increase in the duration and prevalence of breastfeeding during the first years of life.
- Compliance with science-based physiological standards of nutrition in children's pre-school institutions, educational, health and social care establishments.
- Absence of large-scale outbreaks of infectious diseases and food poisoning among the public.
- Reduced morbidity from communicable and noncommunicable diseases among young children.
- Reduced level of primary morbidity in the population.
- Reduced prevalence of complications in the course and outcome of pregnancy.

Monitoring

The programme will be monitored by the departments of the mayor's office: health and social protection of the population; education, culture and sport; the committee for trade and public services; the centre of state sanitary and epidemiological surveillance in the city of Arkhangelsk.

Sources and amounts of funding (in thousands of roubles) for the programme:

	2000	2001	2002	Total for 2000–2002
From the local budget	450	400	400	1250
From enterprises	1500	1500	1500	4500
TOTAL	1950	1900	1900	5750

Annex 1. Table 1. Arkhangelsk Programme – activities, implementing agencies, expenditure

Activities	Implementing agency	Expenditure (thousand rubles)			Sources of funding
		2000	2001	2002	
1. Organizational activities					
1.1. To organize monitoring of health status of the population taking into account nutritional factors	Department of health and social protection of the population of the mayor's office of Arkhangelsk, centre of state sanitary and epidemiological surveillance, Arkhangelsk State Medical Academy	50.0	—	—	Local budget
1.2. To analyse food consumption by the city's population over many years and to determine the principal dietary trends in the population, assessing their correspondence to the recommendations by the Institute of Nutrition of the Russian Academy of Medical Sciences and by WHO	Committee for trade and public services, centre of state san. and epid. surveillance in Arkhangelsk				Local and federal budgets within the approved limits of budgetary allocations
1.3. To analyse the dietary patterns and nutritional status of children over a long period of time, and assess their correspondence to the physiological norms: - for young children - for children of pre-school age - for schoolchildren - for some population groups	Centre of state san. and epid. surveillance; departments of the mayor's office: of health and social protection of the population; education, culture and sports; Medical Academy	—	—		Local and federal budgets within the approved limits of budgetary allocations
1.4. To develop public education programmes in healthy nutrition: - for pregnant and lactating women - for children of pre-school age - for schoolchildren - for secondary and higher educational establishments - for the general public	Centre of state san. and epid. surveillance; departments of the mayor's office: of health and social protection of the population; education, culture and sports; Medical Academy	—	—		Local and federal budgets within the approved limits of budgetary allocations
1.5. To develop training programmes for professional communities: - health professionals at health-care establishments - teachers at pre-school children's establishments - school teachers - faculty members at higher educational institutions - personnel of public catering establishments	Centre of state san. and epid. surveillance; departments of the mayor's office: of health and social protection of the population; education, culture and sports; Medical Academy	—	—	—	Local and federal budgets within the approved limits of budgetary allocations
1.6. To introduce, on a step-by-step basis, rooming-in for breastfeeding mothers and their babies at maternity homes and children's hospitals	Department of health and social protection of the population, heads of health-care establishments	300	300	300	Local budget
1.7. To develop rational diets	Centre of state san. and epid. surveillance; departments of the mayor's office: of health and social protection of the population; education, culture and sports; Medical Academy	—	—	—	Local and federal budgets within the approved limits of budgetary allocations

Activities	Implementing agency	Expenditure (thousand rubles)			Sources of funding
		2000	2001	2002	
1.8. To designate public catering establishments that shall provide hot meals for the low-income population groups	Committee for trade and public services	—	—	—	Local and federal budgets within the approved limits of budgetary allocations
1.9. To consider the issue of subsidizing food for vulnerable population groups (pregnant and lactating women, children)	Departments of the mayor's office: of economics, industrial policy and investments; health and social protection; education, culture and sports; committee for trade and public services	—	—	—	Local and federal budgets within the approved limits of budgetary allocations
1.10. To designate wholesale trade organizations that shall deliver food to child-care, educational and health-care establishments	Committee for trade and public services; departments of the mayor's office: of economics, industrial policy and investments; health and social protection; education, culture and sports	—	—	—	Local and federal budgets within the approved limits of budgetary allocations
1.11. To prepare requests for delivery of local farm produce and to forward them to the department of agriculture of the Arkhangelsk regional administration and to suppliers	Committee for trade and public services	—	—	—	Local and federal budgets within the approved limits of budgetary allocations
1.12. To draft regulations of encouraging and supporting legal entities and natural persons that help provide for healthy nutrition of the population	Centre of state san. And epid. surveillance in Arkhangelsk, Medical Academy	—	—	—	Federal budget within the approved limits of budgetary allocations
1.13. To draft proposals regarding tax privileges to be extended to local food producers and trading organizations, and to submit them to the government of the Arkhangelsk Region	Committee for trade and public services	—	—	—	Local and federal budgets within the approved limits of budgetary allocations
1.14. To perform hygienic evaluation of the quality of exported and domestic baby milk formulas and assess their compliance with health standards	Centre of state san. and epid. surveillance in Arkhangelsk	—	—	—	Federal budget within the approved limits of budgetary allocations
1.15. To conduct a sanitary feasibility study of the manufacture of baby foods (for infants over 9 months old) from natural cow's milk at the OAO "Arkhangelski molochny kombinat" (Arkhangelsk Dairy Plant)	Centre of state san. and epid. surveillance in Arkhangelsk, Medical Academy	—	—	—	Federal budget within the approved limits of budgetary allocations
1.16. To develop a regional policy of advertising healthy nutrition	Centre of state san. and epid. surveillance in Arkhangelsk, Medical Academy	—	—	—	Local and federal budgets within the approved limits of budgetary allocations
1.17. To set up an organizing committee at the mayor's office to coordinate and monitor the implementation of the Programme of Healthy Nutrition	Committee for trade and public services	—	—	—	Local and federal budgets within the approved limits of budgetary allocations

Activities	Implementing agency	Expenditure (thousand rubles)			Sources of funding
		2000	2001	2002	
2. Activities in the field of training and public education and awareness-raising					
2.1. To organize graduate and post-graduate training of teaching staff for child-care and educational establishments in healthy nutrition of the population	Heads of secondary and higher educational establishments (Arkhangelsk State Teachers' Training University, Teachers' Post-Graduate Institute, Arkhangelsk Teachers' School)	—	—	—	Local and federal budgets within the approved limits of budgetary allocations
2.2. To organize graduate and post-graduate training of health professionals in healthy nutrition of the population	Arkhangelsk State Medical Academy, Arkhangelsk Medical College, post-graduate school for mid-level health and pharmaceutical workers	—	—	—	Local and federal budgets within the approved limits of budgetary allocations
2.3. To organize publication of methodological and educational literature on healthy nutrition	Medical Academy	100	100	100	Local budget
2.4. To introduce programmes of training in health-care, educational and child-care establishments	Department of education, culture and sports at the mayor's office	—	—	—	Local and federal budgets within the approved limits of budgetary allocations
2.5. To prepare public catering establishments as centres for basic on-the-job training of public catering personnel in healthy nutrition of the population	Committee for trade and public services, centre of state san. and epid. surveillance in Arkhangelsk	—	—	—	Federal and local budgets within the approved limits of budgetary allocations
2.6. To incorporate issues of healthy nutrition in the programmes of professional health training and to provide for periodic training of personnel working with population groups within the scope of the decree on maternity care	Centre of state san. and epid. surveillance in Arkhangelsk	—	—	—	Local and federal budgets within the approved limits of budgetary allocations
2.7. Systematic coverage in the mass-media of the issues of healthy nutrition of the population	Mass-media organizations, centre of state san. and epid. surveillance in Arkhangelsk	—	—	—	Local and federal budgets within the approved limits of budgetary allocations
3. Activities aimed at increasing local output of food raw materials and foods					
3.1. To develop a plan of leasing out land parcels to farmers and the general public to grow local vegetables, fruit and berries, and to submit this plan to the department of agriculture of the Arkhangelsk regional administration	Department of economics, industrial policy and investments, committee for trade and public services at the mayor's office	—	—	—	Local budget within the approved limits of budgetary allocations
3.2. To organize the purchases of farm produce, mushrooms and berries from the general public	Committee for trade and public services at the mayor's office	—	—	—	Local budget within the approved limits of budgetary allocations
3.3. To increase the output and sales of local farm products	All types of legal entities and natural persons	300	300	300	Funds of enterprises of all forms of ownership
3.4. To study the production of quick-frozen locally grown vegetables, berries and mushrooms at the existing refrigeration facilities	All types of legal entities and natural persons	300	300	300	Funds of enterprises of all forms of ownership

Activities	Implementing agency	Expenditure (thousand rubles)			Sources of funding
		2000	2001	2002	
3.5. To organize production of low-fat milk and dairy products, and health dairy products at the AO Moloko	AO Moloko	—	—	—	Enterprises of all forms of ownership
3.6. To organize production of meats for children's food and school meals taking into account the physiological standards of nutrition	The Arkhangelski Meat Plant	—	—	—	Enterprises
3.7. To expand the range of fish products (frozen half-finished products, scalded preserves, fish pockets) produced by local fish processing enterprises of all forms of ownership	OAO Arkhangelski Fish Processing Plant	—	—	—	Enterprises
3.8. To expand the range of bakery and confectionery products produced by specialized bakeries, including products fortified with iodine and calcium, and products enriched with dietary fibre (products with boltings, from coarse rye flour)	OAO Arkhangelskshleb	—	—	—	Enterprises
3.9. To establish companies to procure and store local farm produce and forest products	All types of legal entities and natural persons	500	500	500	Enterprises
3.10. To ensure appropriate labelling of healthy foods produced by local industries	All types of legal entities and natural persons	—	—	—	Enterprises
4. Measures to ensure marketing of local farm produce					
4.1. To organize centralized supply of local farm produce to children's pre-school establishments and all types of schools, health-care and social establishments	Committee for trade and public services at the mayor's office	—	—	—	Local budget within the approved limits of budgetary allocations
4.2. To organize sales of local farm produce in the markets and shops of the city	Committee for trade and public services at the mayor's office	—	—	—	Local budget within the approved limits of budgetary allocations
4.3. To organize centralized delivery of iodized salt and its sale in shops	Committee for trade and public services at the mayor's office	—	—	—	Funds of all types of legal entities and natural persons
4.4. To ban the supply and sale of breast-milk substitutes to health-care establishments of the city	Department of health and social protection at the mayor's office	—	—	—	Local budget within the approved limits of budgetary allocations
4.5. To forbid sale of breast-milk substitutes and baby milk formulas in general stores without approval of the assortment of goods by the sanitary and epidemiological service	Enterprises and organizations of all forms of ownership, individual private operators	—	—	—	-
4.6. To ensure appropriate labelling of healthy foods sold in shops	All types of legal entities and natural persons	—	—	—	Enterprises
5. Measures to ensure food safety					
5.1. To organize factory control of the quality of food raw materials and foods at all stages of production, handling, storage and sale, including laboratory control	Enterprises of all forms of ownership	100	100	100	Enterprises

Activities	Implementing agency	Expenditure (thousand rubles)			Sources of funding
		2000	2001	2002	
5.2. To ensure that sale, storage and handling of food raw materials and foods is conducted in compliance with the sanitary standards and regulations	Enterprises of all forms of ownership	—	—	—	Enterprises
5.3. To issue compliance certificates for food raw materials and foods subject to availability of health reports certifying that the product in question meets sanitary standards and rules	Centre of standards, metrology and certification	—	—	—	Funds of institutions
5.4. To organize sale of products of animal husbandry, apiculture and forest products only in the markets having their own sanitary control laboratories	Committee for trade and public services of the mayor's office	—	—	—	Local and federal budgets within the approved limits of budgetary allocations
6. Measures aimed at organizing therapeutic and disease-preventing diets					
6.1. To organize production of foods for therapeutic and disease-preventing diets by local food industries	OAO Arkhangelskhele (Bakery Company), OAO Moloko (Dairy Company), OAO Myasokombinat Arkhangelski (Meat Plant), AO Arkhangelski Rybokombinat (Fish Processing Company)	300	300	300	Enterprises
6.2. To organize sale of foods for therapeutic and disease-preventing diets in stationary shops	Committee for trade and public services	—	—	—	Local budget within the approved limits of budgetary allocations
To organize supply of foods for therapeutic and disease-preventing diets to hospitals in the city and to specialist disease-preventing establishments	Committee for trade and public services	—	—	—	Local budget within the approved limits of budgetary allocations

Annex 2

Programme

Monday, 22 November, 1999

Chairpersons: A. Baturin, L. Koudrya

14.00 – 14.20 Opening of the meeting: representatives of the city of Arkhangelsk and the RCPM

14.20 – 15.00 **Plenary session**

General discussion of the document, chapters 1-4, presentation of the chapters by the authors

15.00 – 17.30 **Working groups:** group discussion of chapters 1-4. The authors of the chapters present comments to the plenary session.

16.00 – 16.30 Break

Tuesday, 23 November, 1999

Chairpersons: T. Kamardina, N. Danchin

9.30 – 12.00 **Plenary session**

Summing up discussions in the working groups. General discussion of the document, chapters 5-7, presentation of the chapters by the authors.

11.00 – 11.30 Break

12.00 – 13.00 **Working groups:** discussion of chapters 5 – 7, recommendations for developing a regional policy of healthy nutrition of the population, monitoring and evaluation. Conclusions.

13.00 – 14.00 **Lunch**

14.00 – 17.30 **Working group discussions continue**

15.30 – 16.00 Break

Wednesday, 24 November, 1999

Chairpersons: A. Robertson, O. Kalev

9.30 – 11.30 **Plenary session**

Summing up of working group discussions. The authors of the chapters present all the comments to the plenary session. Further plans regarding the drafting of the document.

11.30 – 12.00 Break

Annex 3

Participants

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11. Aileen Robertson	Acting Regional Adviser for Nutrition, Programme for Nutrition Policy, Infant Feeding and Food Security, Lifestyles and Health Unit, WHO Regional Office for Europe	8 Scherfigsvej DK-2100 Copenhagen, Denmark Fax: 45 39 17 18 184 e-mail: sal@who.dk

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16. Lydia Kolominova	Leading expert for trade and nutrition, Arkhangelsk city mayor's office	163013, Arkhangelsk

Participants in the meeting representing the mayor's office, Troitski prospekt 49, Arkhangelsk, 24 November, 1999

1. Ye. Abramova	Regional newspaper "Pravda Severa", correspondent	
2. N. Barbolin	Head of administration, Majskaya Gorka District	Arkhangelsk, Troitski prospekt, 49
3. L. Yegumnova	Head of administration, Lomonosovski District	Arkhangelsk, Troitski prospekt, 49
4. S. Zhigalov	Chairman of the Standing Committee for Humanitarian and Social Issues at the City Soviet	Arkhangelsk, Troitski prospekt, 49
5. G. Zonova	Head of administration, Oktyabrski District	Arkhangelsk, Troitski prospekt, 49
6. R. Korotenkova	Head of department of trade and public services at the mayor's office	Arkhangelsk, Troitski prospekt, 49
7. T. Kulimaniva	Head of department of education, culture and sports at the mayor's office	Arkhangelsk, Troitski prospekt, 49
8. V. Langueman	Head of administration, Isakogorka and Tziglomenski Districts	Arkhangelsk, Troitski prospekt, 49
9. V. Loseva	Head of the anti-monopoly committee	Arkhangelsk, Troitski prospekt, 49
10. G. Malysheva	Deputy head of administration, Maimaksan District	Arkhangelsk, Troitski prospekt, 49
11. V. Panteleyev	Chief of staff of the mayor's office	Arkhangelsk, Troitski prospekt, 49
12. V. Popova	Head of administration, Severny District	Arkhangelsk, Troitski prospekt, 49
13. Yu. Rasulov	Deputy head of administration, Solombala District	Arkhangelsk, Troitski prospekt, 49
14. A. Rybinskaya	Deputy head of department of economics, industrial and tax policies at the mayor's office	Arkhangelsk, Troitski prospekt, 49
15. O. Semoushina	Expert of the health department at the mayor's office	Arkhangelsk, Troitski prospekt, 49
16. A. Chebykin	Head of administration, Factoria-Varavino District	Arkhangelsk, Troitski prospekt, 49

Annex 4

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References

Glossary of terms used in the document.

Annex 5

Analysis of the WHO Baby Friendly Hospital initiative in Arkhangelsk

Nina Kondakova, Regional Breastfeeding Initiative Coordinator in the Arkhangelsk Region

The following activities have been carried out under the Arkhangelsk Regional Programme for Breastfeeding Protection, Promotion and Support (1996–1998) within the framework of the Baby-Friendly Hospital Initiative (BFHI) in Arkhangelsk project:

- a Coordination Council for Breastfeeding was set up in Arkhangelsk in 1998;
- a paper was published describing the regulation of activities in breastfeeding support in Arkhangelsk and restrictions on breast-milk substitutes following the main points of the International Code of Marketing of Breastmilk Substitutes;
- information on breastfeeding has been published in newspapers and special booklets;
- conferences have been held in the towns of Kotlas and Novodvinsk;
- programmes and educational courses on breastfeeding problems have been arranged, and the following have been trained each year:
 - 50 obstetricians and paediatricians (40 academic hours)
 - paediatricians from children clinics (4 academic hours)
 - 10 neonatologists (6 academic hours) within the framework of Postgraduate Faculty of the Arkhangelsk Medical Academy
 - 30 medical nurses and obstetricians during practical work at colleges of the Health Care Department of Arkhangelsk region
 - 50 students from the Social Workers' Faculty of Arkhangelsk Medical Academy (40 academic hours)
 - 75 students from the Paediatric Faculty and 140 students from the General Practice Faculty (4 hours).

The Promotion of Breastfeeding Initiative in Arkhangelsk in 1996–1998 faced the following difficulties:

- up to 1999 maternity home regulations restricted mothers and babies from staying in the same ward;
- the authorities underestimated the danger from the prevalence of breast-milk substitutes in treatment institutions – much has now been done to prevent their use.

In 1999 an agreement was published with the sanitary-epidemiological control bodies.

Dr L.A. Sokolova, head of the State Sanitary-Epidemiological Control, contributed actively to it. The WHO workshop on Breastfeeding and Healthy Eating in Pregnancy and Lactation, held in Arkhangelsk from 5 to 8 October 1998, was very helpful in this connection.

The BFHI initiative is developing at present. A Regional Coordination Committee for Breastfeeding Support has been established, which includes participants from the Arkhangelsk State Medical Academy and the Health Care Departments of Arkhangelsk city and region. A programme on breastfeeding for 1999–2000 was worked out for the Arkhangelsk region and is being carried out successfully.

Summary of progress on breastfeeding in Arkhangelsk (1999)

In August 1999 an application was sent to UNICEF for evaluation of Municipal Hospital No. 7 under the Baby-Friendly Hospital Initiative. In the hospital's maternity department (head doctor V.L. Kabakov), 97% of newborns start breastfeeding immediately after delivery and 90% stay with their mothers in the same ward. All 10 steps of successful breastfeeding are applied in this Department. Early neonatal mortality at the hospital fell from 4.1 per 1000 babies in 1996 to 2.8 in 1998. More babies are being breastfed during their first year: 58% were breastfed for 4 months in 1998 as against 39% in 1996, and 49% were breastfed for 6 months in 1998 as against 29% in 1996 (data from Children Consultation of the Municipal Hospital No. 7).

Other hospitals are not so advanced in BFHI, but still their achievements are very good.

Consultations on breastfeeding are arranged at children's clinics and women's consultations. Between 1997 and 1999, staff at the Department of Neonatology and Perinatology, Arkhangelsk State Medical Academy, analysed the morbidity of 123 children from Solombal district aged under 2 years who were being fed differently. The frequency of anaemia, respiratory infections, pneumonia, bronchitis, conjunctivitis and other infections were definitely lower among the children who were fed with breast-milk during the first 6 months than among children who were fed in a different way (Table 1–4). However, only a few children are fed with breast-milk during the first 6 months, which confirms the need for the Breastfeeding Initiative promotion.

Table 1. Risk of anaemia, by feeding method (%)

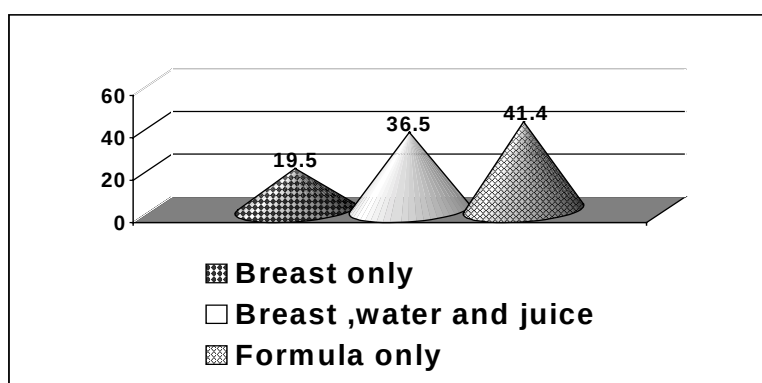


Table 2. Risk of bronchitis and pneumonia, by feeding method (%)

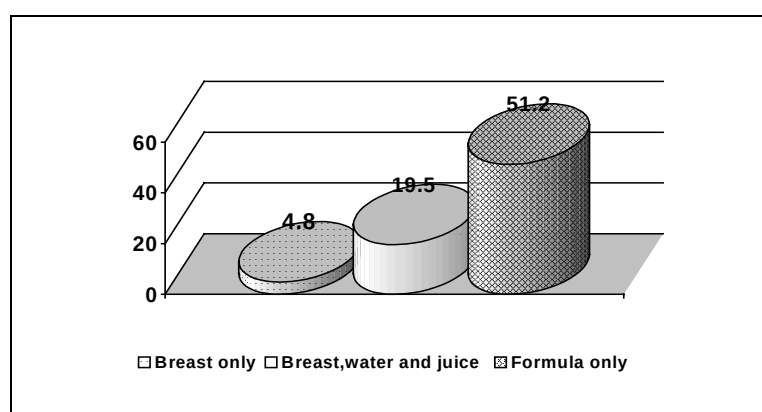


Table 3. Risk of conjunctivitis, by feeding method (%)

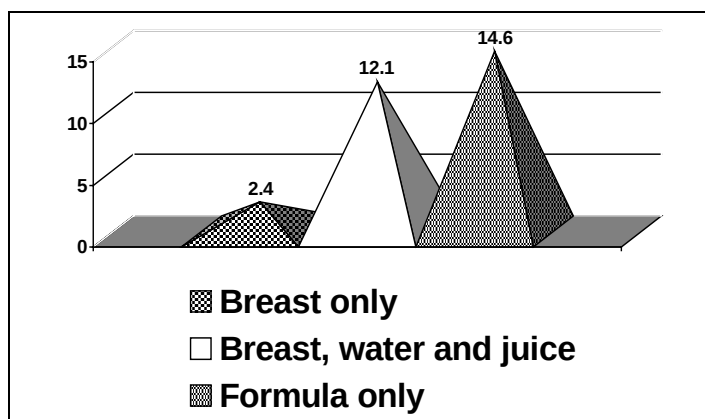
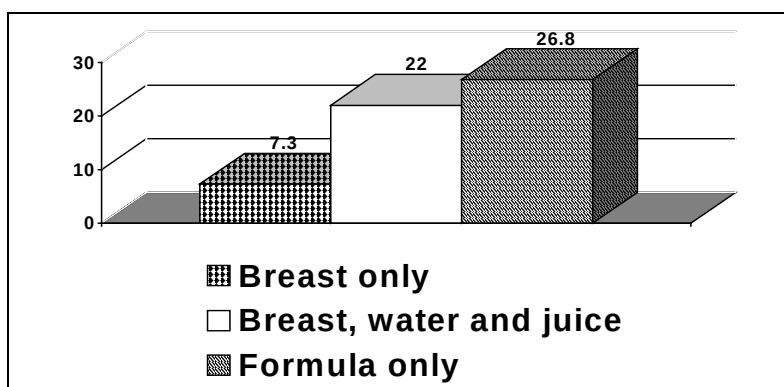


Table 4. Risk of infections, by feeding method (%)



We are grateful to Aileen Robertson, Acting Regional Adviser for WHO's Programme for Nutrition Policy, Infant Feeding and Food Security, who has done so much to contribute to the promotion of the Breastfeeding Initiative in the Arkhangelsk region.

It is very important to hold the following seminars for specialists in the Arkhangelsk region:

- "Breastfeeding and healthy eating in pregnancy and lactation";
- "Consultation Course in Breastfeeding" with the help of trainers in Arkhangelsk and Murmansk and an international WHO expert (course director);
- "Practice of Careful Delivery" course with Ms. Anne Hojmark Jensen (Denmark);
- Feeding of children older than 6 months.

We should be grateful for any help WHO could give us in this connection.

Messages from Arkhangelsk supporting the initiative

АРХАНГЕЛЬСКАЯ ГОСУДАРСТВЕННАЯ МЕДИЦИНСКАЯ АКАДЕМИЯ

ARKHANGELSK STATE MEDICAL ACADEMY



№ _____ 15.12.99 г

Dr Aileen Robertson
WHO Nutrition Copenhagen

Dear Dr Robertson,

We are very grateful to you for your visit and work that helped us greatly in the development of a coalition for the problems solution in healthy eating of Arkhangelsk inhabitants.

We have held one more meeting on this matter. Now we start working out "Programme of Health Eating in the Arkhangelsk Region".

Wishing you health and success

Ludmila Kudrya

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